

DOCUMENTED ALTERATION OF FEDERAL HEALTHCARE CLAIMS

Optum / VA Community Care Network

A Relator's Disclosure

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Submitted to Morgan Verkamp LLC and Susman Godfrey LLP

Identical material provided to both firms. Each firm informed of the other.

Original source. Nothing filed to date.

WHAT THESE THREE FILES SHOW

Three exports of one Optum VA Community Care Network account, taken across twenty-one months. They do not agree with each other.

PART ONE — THE DENTAL MATTER Two years of work. This is the case as it has been understood.

1 A CLAIM WAS DELETED AND REBUILT

Dr. James Russell Green IV, \$6,568.00, surgery of 13 September 2022. Present in the first export with identifier ending 0000, submitted 10/18/2022. Absent entirely from the second export, a complete 137-claim file. Present again in the third with identifier ending 0001, submitted date moved to 10/01/2024, and a facility name added that was not there before. The procedure contents are identical in both appearances. The third export is filtered to paid claims. It was paid.

2 A PROCEDURE WAS BILLED THAT WAS NEVER PERFORMED

The companion claim, \$8,217.00, carries four alveoloplasty line items at \$386.00 each — \$1,544.00 for bone smoothing that was not done. Corrective alveoloplasty was performed by a different surgeon nearly two years later. Those four line items are present in the first export, absent from the second, and present again in the third.

3 TWO IDENTICAL DENTURE CLAIMS WERE BOTH PAID

Same service date, same \$4,775.00, same procedure codes, same line-item amounts, submitted fifty-two days apart. Both appear in the paid-filtered export. \$9,550.00 for one set of dentures. Every other duplicate set in this record produced exactly one payment.

PART TWO — THIS IS NOT DENTAL Found on 21 July 2026. It is why this correspondence is a day later than intended.

4 THE SAME IDENTIFIER CHANGE, ON A HOSPITAL BILL

Tampa General Hospital, \$19,648.23, date of service 6 June 2021. The identifier ends 0000 in the first export and 0001 in the second. Date of service, submitted date, amount and provider are unchanged. Different provider, different service line, no connection to the dental matter — the same signature as Finding One.

5 AN ENTIRE HOSPITAL ADMISSION IS GONE

The September 2021 admission — ten claims across three days, including \$40,762.62 to Tampa General Hospital — appears in the first export and nowhere in the second. The date 09/06/2021 occurs five times in the first file and zero times in the second. The November 2021 admission is likewise absent. Twenty-five claims in total are present in the first complete export and absent from the second. These are pancreatitis hospitalisations, unrelated to any dental provider.

6 THE PATTERN RUNS THROUGH SMALLER CLAIMS TOO

InPhyNet Contracting submitted the same date of service four separate times at \$2,337.00 each, across six and a half months. Two of those four are present in the first export and absent from the second. The

first export returned 135 results and the second returned 137, while at least twenty new claims were generated between the two dates.

WHAT SEPARATES THESE FINDINGS

A provider resubmitting an unpaid claim is ordinary billing. A record changing identity, or disappearing outright, between two exports of the same account is not something a provider's office can do. It happens inside the platform.

A NOTE ON HOW THIS CAME TOGETHER

This is how this case has been for me. It is not something I do for a living, and after two years I am still pulling more out of it — the second half of the findings above surfaced the morning this was sent.

I also had to live through part of it while documenting it. A year ago I did not have the scope of information I have now. A year ago this did not make sense, and what I put in front of people was not professional. That is no longer the case.

What follows is organised so the provable material comes first and the account of the person bringing it comes last. Nothing in the back is load-bearing for anything in the front.

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PURPOSE AND CONTENT

WHY BOTH FIRMS

I am bringing this to two firms at once, and each of you knows I am approaching the other. You are receiving identical material. I am not shopping this quietly or playing offices against each other.

The reason is straightforward. One of you has been down this road against this company before and knows the terrain. The other does not let go of something once it has hold of it. This case needs both of those things, and I would welcome the two of you considering it together.

I am also aware that a relator does not get to assign counsel to each other, and I would not presume to try. But if I am anywhere close to right about the number of separate places this is exposed, then forgive me for speaking out of turn against all of your experience when I say it plainly: this could be big. Bigger than the piece any one of us is looking at. That is the only reason I am asking you to consider it together — the surface is large enough to justify the request.

TIMING

One point of timing, raised first because it is the most perishable thing in this document.

The Optum contract with the VA Community Care Network is in its sunset and under recompetition. A matter that lands while the government is still deciding whether to re-award the same contract to the same party is worth more than one that lands after signature.

A SECOND POINT, FOR ONE OF YOU IN PARTICULAR

The Poehling matter turned on a knowledge defense. The government already knew, therefore nothing was concealed.

What I hold is the inverse. A claim removed and re-entered after submission with a changed identifier and a shifted date. A portal lockout while I was in the middle of documenting it. And then, in writing to a sitting United States Senator, the flat assertion that my account was never locked at all.

That is not a knowledge problem. That is concealment with a timestamp on it — and a second act of concealment, on letterhead, denying the first.

THIS DOCUMENT

This document is the match. The full powder room is behind that door. Don't bring the match.

What you have here is the case: how it started, who built it, what the fraud looks like, and how far it reaches. And it reaches a great deal further than I understood when I began. What started as a question about my own billing has turned into something with an unusual number of separate points of exposure — the altered claims, the concealment, a written record to a Senator that does not hold, a detection system dark for sixteen months, and federally required obligations nobody outside the company ever audits. Any one of them is a thread. Together they are the reason this document exists.

It is still deliberately data-light. The facts are stated, with representative proof attached in places rather than the full record, because I have learned that handing someone everything at once is how a case gets

set aside rather than read. What is toward the front is what I can prove. What is toward the back is where I am reaching, and I mark it plainly when I do.

There is significantly more. Full records pulls and my own VA Blue Records, two versions, more than anyone wants to know about me. It includes records pulls, secure messages spanning years, a congressional response, and a documented VA file running back more than a decade. Any of it is available immediately on request. More facts, fewer words.

The second document covers me. Background, how I came to be the person holding this, and the parts of the story that explain the origin but do not prove the fraud. I have kept it separate on purpose. Nothing in it is load-bearing here. If you want it, it exists. If you never read it, this document still stands on its own.

That separation is intentional throughout. What I can prove is in front of you. What I believe but cannot prove is held back, marked as belief, and available to counsel directly rather than written into a document that travels.

Nothing has been filed. No qui tam complaint, no tort claim, no lawsuit. I made earlier inquiries to a few other firms before I had assembled this, none of which resulted in engagement, and I would rather name that here than have it surface later.

CLAIM AND SUPPORT

WHAT I FOUND

I had reasons to pull my own Optum records. What I found was damning.

One example: payment for a full set of dentures, unplanned, months before my surgery. That is not how the sequence works. Alongside it were changed item record IDs, and repeat submissions of the same claims until some of them took. A separate office submitted for a full set of permanent dentures a month after I had already switched away from them, and again a month after that. Both were paid.

The most personal is the billing that followed the surgery itself.

I went under with the surgeon and a contract anesthesiologist present. I woke an hour later, halfway through the extraction, and the teeth were being removed in a manner that caused damage. No irrigation was performed. No alveoplasty was performed. The surgeon called in antibiotics at high levels and no pain medication at all. He was on a plane with his family within the hour. Another office had to redo the suturing, pulling the gums back over the sharp bone left behind. They knew what they were doing as well.

I have had fifteen cases of pancreatitis. Each one is like a ten day slow-motion gang initiation beating. I say that so the next part is understood in scale: bone and fragments puncturing the gums, with thrush on top of it, is a special kind of hell.

Both the denture office and the VA dentist marked the alveoplasty as performed. It was not. It was finally done nearly two years later, while VA doctors told me the bone piercing through my gums was natural. Every office involved, VA and Community Care alike, took x-rays. Every professional in that chain is trained to see the absence of an alveoplasty.

I called Optum to gather details and was told the bills had never been submitted. I logged into the portal myself and found them. One in excess of six thousand dollars, one in excess of eight thousand. Both paid. Both claiming the alveoplasty had been performed, by two different methods of claiming it. There is no explanation on the record for why two were submitted at all.

I downloaded everything and filed two fraud reports with Optum. Both are later referenced in Optum's written response to Senator Moody's office — a response that is itself deceptive.

I was already on Optum's radar before this. Filing made me watched.

Over the following months I pulled a second copy of my records. The six thousand dollar claim had been removed. Then it reappeared, dated two years after the surgery. I was locked out. I waited, logged back in a final time, and captured a screenshot of the corrected date before losing access again. That lockout has been permanent. I have not attempted access since, deliberately, so that no one can characterize this as unauthorized access.

The surgery happened on Tuesday, September 13, 2022. I was there. Yet nearly every date in the record falls on the eleventh or the eighteenth. Both are Sundays. Neither is the day the surgery occurred. Any record that does not say the thirteenth is wrong on its face, because the thirteenth is the day it happened, and it was the worst Tuesday of my life. Which tracks. Tuesdays.

WHAT I DO NOT KNOW HOW TO SAY PROPERLY

I do not have the vocabulary for this part. I am going to describe what I am looking at in plain terms and let you put the right words on it.

A claim in my file was deleted after submission and re-entered under a new identifier with the date moved forward two years. I know what that is at a systems level. It is a write operation against a live record, performed by someone with access above the provider tier, in a way that did not preserve the original.

Here is what I do not know how to say properly.

If a company can reach into a submitted federal claim and change what it says, then the record is not a record. It is a position the company is currently taking. It can be revised. And if it can be revised, then whatever the government is looking at is whatever the company decided the government should see.

I understand that organizations keep internal accounting that differs from what they report, and that there are legitimate reasons for that. What I do not understand is how a system permits the reported version to be rewritten after the fact, silently, and still be treated as the authoritative record of a federal payment.

I am not accusing anyone of keeping two sets of books. I have never seen their books and I would not know what I was looking at if I had.

I am saying something narrower and, I think, harder to argue with. I proved they can change the record, and I proved they did. Everything downstream of that is a question about what else was changed, and I have no way to answer it.

WHY NOBODY CAUGHT IT

The alteration was not detected by a control. It was detected by me.

I found it because I pulled my own file three times over two years and compared the copies. There is no reason anyone at that company would expect a patient to do that. Nothing in their process assumes it.

Meanwhile, the VA's Program Integrity Tool, the mechanism built to catch this exact category of thing, was offline for sixteen months. My alterations fall inside that window.

So the only functioning fraud detection operating on my account, during the period my account was altered, was a patient with a habit of keeping his downloads.

I am one person with access to one account, and only because it was mine. I am not asserting anything about anyone else's records. I am telling you that the reason this one surfaced has nothing to do with any system working, and everything to do with an accident of who the patient happened to be.

ONE EXHIBIT AVAILABLE TODAY

Independent of anything still arriving, one item is retrievable now: a message from the mental health provider, located in my Blue Button export.

Also in that export are messages I sent during the worst period, characterized in the record by staff as offensive — sitting directly alongside the contemporaneous account of what I was actually reporting at the time. Their own labeling is the corroboration.

MY QUALIFICATION TO READ IT

I spent twenty-five years in this work. MCSE in 1997 as a systems administrator, MCDBA and MCSD in 2004 as a database and C# developer, and I am closing a master's in cyber intelligence three years after starting with no degree at all.

I say that for one reason. When I looked at that portal, I was not looking at it as a patient. The application was not broken. It was not glitching. It was functioning exactly as designed — and what it was designed to permit was exactly what I found: claims stripped of their original identifiers, re-dated, and resubmitted until they take.

The altered records also generated visible errors in the interface itself. That tells me the people doing this were not skilled at this level of deception. Some of them appear not to care whether it shows, as though it does not matter what anyone thinks, because they will do what they want.

That is my own professional read, and it is the one I would give under oath.

THE LOAD-BEARING POINT

None of this matters unless the system itself reaches the parent. I believe it does.

What follows is my own research. I make no claim to your level of experience — only to having done the work.

The False Claims Act knowledge standard, 31 U.S.C. § 3729(b)(1), defines knowing to include actual knowledge, deliberate ignorance of the truth or falsity of the information, and reckless disregard — and requires no proof of specific intent to defraud. Any one of the three is sufficient. That forecloses the position that a platform was purchased and its behavior unexamined.

The liability provisions, § 3729(a)(1), reach conduct that knowingly presents or causes to be presented a false claim, and that knowingly makes, uses, or causes to be made or used a false record material to a false claim. The reverse false claims provision at (a)(1)(G) reaches knowing concealment or improper avoidance of an obligation to pay the government. The phrase causes to be presented is the operator hook — it reaches an entity that did not submit the claim itself.

Materiality is defined at § 3729(b)(4) as a natural tendency to influence, or capacity to influence, payment.

On reaching the parent: *United States v. Bestfoods*, 524 U.S. 51 (1998) establishes that a parent is directly liable for its own conduct in operating a facility, as distinct from derivative liability for a subsidiary's conduct. Ownership does not create liability. Operation does. Direct participant liability is addressed in *Steiner Electric Co. v. Maniscalco*, 2016 IL App (1st) 132023, and *Northbound Group, Inc. v. Norvax, Inc.*, 795 F.3d 647 (7th Cir. 2015).

I ask counsel to verify the current statutory text before anything is filed. I raise it because the operator question is the one that determines whether this reaches UnitedHealth Group, and I believe the answer is yes.

THE PLAN THAT STARTED THIS

WHO COULD HAVE DONE THIS

I am not asking you to accept that it was her because I say so.

The conduct required four things at once. Deep institutional knowledge of how the VA handles a patient's file. Proximity to power sufficient for management to stand back. A method already demonstrated to work. And a motive.

Those four converge on one person.

THE INSTIGATOR

She is a thirty-year retired VA nurse. Daughter of a high-ranking officer who commanded a base, raised inside the expectation of the perfect military family.

I am not offering a clinical diagnosis. I am telling you what thirty years in that system gave her: the plan, the contacts, and the working knowledge of exactly how the VA responds to what is in a veteran's file.

She used Optum to push VA providers toward outcomes that would end a veteran. I still hold an email from her that demonstrates the intent.

THE INSIDER

The mental health provider in Tampa gave me my diagnosis in 2015 — a diagnosis that expressly recorded no traits for self-harm or self-deletion. He was later recruited.

That is the man who used his own documentation against the patient it described.

THE CHAIN

The instigator. The provider who turned my diagnosis into a weapon. And the company that told VA management to stand back while it happened.

I have every link of that chain, end to end. Each part has been researched, validated, and tested against the effects of my own actions — and each time, the result changed in the way the chain predicted.

ONE ITEM I HAVE NOT VERIFIED

I believe veteran messages to mental health and medical staff route to a central mailbox read by multiple providers and nurses. If that routing holds, the position that this pattern went unseen for years is not credible.

I have not confirmed it and I am not asserting it. I flag it because it is checkable and because I would rather hand you an open question than a bad claim.

THE WITHDRAWAL

I have told the providers directly that I am withdrawing my complaints to the State of Florida. They did not want any of this. My dental work is still unfinished six years on, and I do not hold that against them or against VA management.

I did it before anyone asked me to, and I mention it here only because it is the clearest statement I can make about what I am after. I released the people I could have pursued.

The VA is the closest thing to family I have. I know how that sounds to some people. You did not know my parents, and no one needs to know them today.

WHY THIS IS BIGGER THAN A BILLING CASE

THE RACE TO THE BOTTOM

UnitedHealth Group works with roughly a hundred thousand providers. Some portion of them participate in this. The rest cannot do anything about it. That is not a moral distinction I am drawing — it is a structural one. When the largest payer sets the floor, everyone else has to meet it to stay in business.

I asked two offices directly — the surgical office and the denture office — whether they played the game. Both understood exactly what I meant before I finished the sentence.

I speak only for myself and for conversations I have had with a number of people over time. But this is an open secret. Anyone can ask their own provider.

It is also how the rest of it was done to me. Coercion runs on the same leverage.

THREE FACTS AND THE QUESTION THEY RAISE

The VA's Program Integrity Tool — its fraud detection capability for community care claims — was offline for sixteen months.

Contractors operating under federal contract carry records retention and integrity obligations throughout, regardless of whether anyone is watching.

The alterations to my records occurred inside that window.

I am not asserting a cause. I am saying that is precisely what an investigation with subpoena power exists to answer.

THE CONGRESSIONAL RESPONSE

There is a documentary record of how this company answered a United States Senator, and I would ask you to read it more closely than any other single item in this package. Everything else here is my account of what happened to me. This is the company speaking for itself, in writing, to Congress. It is the one place where they, and not I, chose the words.

In response to Senator Moody's inquiry on my behalf, Optum sent two letters, dated March 30 and May 5, 2026. Both purport to address my fraud report and my quality-of-care concern. Neither engages the substance of either one. Read carefully, they do something worse than dodge. They build a wall out of authorities that do not belong to the party invoking them.

Start with what the letters concede, because the concessions are useful. Optum confirms its Payment Integrity Department received my fraud report. It confirms its Clinical Quality department received two separate quality-of-care referral forms, dated March 27 and September 10, 2024. So the existence of the reports is not in dispute. They were made, they were received, and the company admits it to a Senator.

Then the wall goes up.

On the fraud report, Optum states that a veteran who submits a fraud tip is not considered directly involved in the investigation and has no visibility into it. Sit with the structure of that. It is not an answer

to the Senator's question. The Senator asked whether I, as the originator of the complaint, was entitled to know anything. Optum converted a question about my standing into a rule that guarantees I will be told nothing. The answer to "is he allowed to know" became "no one like him is ever allowed to know."

On the quality-of-care concern, the same move. Confirmation of receipt, followed by the statement that a submitter is not notified and that outcomes are not shared. And then the line that removes any doubt about the intent of the correspondence: this single letter, they write, will serve as the only formal communication regarding either matter. Not the first. The only. They confirmed to a United States Senator that they received reports of fraud and of endangered care, and in the same breath foreclosed, permanently and in writing, any possibility that the person who made those reports would ever learn what became of them.

That is not a process failing. That is a process working exactly as designed, described in the company's own words. A report goes in. Nothing comes out. The person who filed it is structurally sealed off from the result, forever, by policy.

Now the part that I would put in front of your best regulatory mind, because I believe it is the sharpest edge in this entire document.

The authority Optum cites for all of this withholding is 38 C.F.R. Section 5705.

Section 5705 is a Department of Veterans Affairs regulation. It governs the confidentiality of VA medical quality-assurance records — internal peer-review and quality-improvement materials generated within the VA system. It exists to protect the candor of the VA's own internal quality reviews. It is a shield the VA holds, for the VA's own records.

Optum is not the VA. Optum is a private government contractor. And a fraud complaint is not a medical quality-assurance record.

So consider what has actually happened on the page. A private company, answering a Senator, reached across the fence, picked up a confidentiality regulation that belongs to the federal agency it contracts with, and held it out as its own authority to refuse a fraud complainant any information about a fraud complaint. It invoked a VA quality-assurance shield to bury something that is neither a VA record nor a quality-assurance matter. Whether that is a misunderstanding of the regulation or a deliberate misuse of it is not for me to say. But it is checkable in an afternoon by anyone who reads the regulation, and I would ask that someone do exactly that. Because if a company is willing to misapply a federal confidentiality statute to a Senator, in writing, to avoid answering for a fraud report, that tells you something about what it is willing to do where no Senator is watching.

Then there is the flat falsehood, which needs no regulatory analysis at all.

The March 30 letter asserts that Optum's technology department confirmed my portal account was active and not locked as of March 27, 2026.

I have three record pulls taken across different dates, and a documented lockout, that say the opposite. This is not a matter of interpretation or of competing recollections. It is a specific factual representation, made to a United States Senator, that my own records directly contradict. Someone wrote that sentence. Someone reviewed it. Someone signed it. And it was not true.

Finally, set the two institutions side by side, because together they say what neither says alone.

Optum told the Senator, in substance, that the matter was closed to me by design — that I was not entitled to know anything and that its single letter would be the last word. The Department of Veterans Affairs, responding to the same Senator on April 27, 2026, stated that its community care analyst had repeatedly tried to reach me to discuss my payments, my dental treatment, and the records on file.

One party told a Senator the door was closed and would stay closed. The other told the same Senator it had been knocking on that same door trying to talk to me about the very records at the center of this. Both cannot be the true posture. At least one of those representations to Congress does not hold, and the gap between them is not something I created. It is on their own letterhead.

I am not going to tell you what any of it means. I was not in the room where either letter was written, and intent is precisely what an investigation with subpoena power exists to establish. I am placing the letters in front of you because they are dated, they are signed, they contradict my records and each other, and they lean on an authority the company invoking it does not appear to hold. They are the company's own words, and they are the reason I stopped thinking of this as a billing dispute.

THE WINDOW, AND WHAT WAS OWED DURING IT

Three facts sit next to each other.

The VA's Program Integrity Tool, its fraud detection capability for community care claims, was offline for sixteen months.

The alterations to my records occurred inside that window.

And a contractor operating under federal contract carries records retention and integrity obligations the entire time, whether anyone is watching or not. FAR Subpart 4.7 requires retention of records related to contract performance and names backup files explicitly as a category that must be kept. Electronic records must be held on reliable media, an audit trail must be maintained when data moves between systems, and the records must be complete and accurate.

Separately, 31 U.S.C. Section 3729 is not the only statute in play. Under 18 U.S.C. Section 1519, knowingly altering, destroying, or concealing a record with intent to impede a federal investigation is a felony.

So the question is not whether obligations existed during those sixteen months. They did, in writing, continuously. The question is what happened to the records during a period when the mechanism designed to catch exactly this was dark.

I am not asserting a cause. That is what an investigation with subpoena power exists to determine.

WHAT I WOULD ASK COUNSEL TO PRESERVE FIRST

This is a recommendation, and I make it as someone who spent twenty-five years administering systems rather than litigating them.

What is already established in my own records is that claims were altered after submission. That behavior is documented. It is the thing I can prove.

A party that alters submitted records has, by definition, shown a willingness to change what is retained. The federal retention framework anticipates exactly this, which is why it requires audit trails and completeness rather than trusting the operator's word.

My concern is narrow and I want to state it as narrowly as it is. Contract-required backups and retained electronic data should be preserved, and their integrity verified, early, before anything degrades further in the ordinary course. I am not claiming anything has happened to them. I have no evidence of that and I am not asserting it.

I raise it because the party in question has already demonstrated the relevant behavior at the record level, because there is no standing government mechanism that adversarially tests a contractor's retained data, and because preservation is one of the few things in this that gets harder with time rather than easier. The verification only happens when someone with the authority to demand it does so.

WHAT THEY DO WHERE PEOPLE CAN SEE

Before I ask you to consider the part of this that no one reports on, look at what is already documented in the parts people can see. This is not my case. These are matters of public record, across the populations this company touches, and none of it depends on me.

Veterans. My own file is the immediate example. Beyond it, the VA Office of Inspector General has published findings on improper community care payments and uncollected recoveries in the program Optum administers.

Medicare. The Department of Justice intervened in whistleblower litigation alleging UnitedHealth made patients appear sicker than they were to raise reimbursement, the government asserting Medicare would have paid on the order of two billion dollars less absent unsupported diagnosis codes. Separately, families brought a class action alleging the company used an algorithm called nH Predict to cut off post-acute care for elderly patients, a tool the complaint alleges the company knew carried roughly a ninety percent error rate, with denials reversed on appeal the great majority of the time, used in reliance on the fact that few patients ever appeal.

Medicaid. In 2026 Massachusetts sued UnitedHealthcare, alleging it inflated the sickness of low-income seniors to draw at least one hundred million dollars in improper payments, and that even after downgrading members it never told the state it had been overpaid or offered to return the money.

Children. The Government Accountability Office has documented over a billion dollars in duplicate Medicaid and CHIP coverage paid to managed care organizations, and HHS OIG has documented unallowable CHIP payments made for beneficiaries carrying multiple identification numbers, the same category of identifier irregularity that appears in my own records.

The federal workforce and military families. Here the public documentation thins, and not because the exposure is smaller. The same company, running the same billing infrastructure, administers coverage touching federal employees and military retirees. The question there is not whether the incentives change. It is who has looked.

That is the visible layer. Every item is public and attached to a population of human beings, and every one of them became known the same way.

A patient kicked out of a nursing home has a family. A senior coded sicker than she is has a state watchdog. A veteran has a Senator. Harm to a person creates a witness, and a witness creates a record.

Every matter above exists because someone was standing in the harm and able to say it happened to me.

That is the line the next section crosses.

Infrastructure has no witness. A neglected backup site has no family. An untested failover files no appeal. A data center shared across programs that are not permitted to share calls no reporter. If the same company that does all of the above where people can see it is also cutting corners where no one can, there is no patient to notice and no family to sue. That harm stays invisible until the day it is not, and by then it is not a denied claim, it is a lost record for a great many people at once.

So I am asking you to carry the pattern across that line. The willingness is documented. The only open question is whether it stops at the places where someone would notice.

THE INFRASTRUCTURE QUESTION

I want to raise one more area, and I want to be as clear about its limits as I have been about everything else. I do not have visibility into these systems. I am not asserting that anything in them is deficient. What I am doing is asking a question that I am qualified to ask, and recommending you put it to people who can answer it.

A contractor administering federal health programs at this scale is required to maintain a large and expensive body of infrastructure. Alternate processing and storage sites. Tested disaster recovery. Continuity-of-operations capability. Separation of data between programs that are not permitted to commingle. This is not discretionary and it is not free. It flows from FISMA (44 U.S.C. Sections 3551 to 3554), OMB Circular A-130, NIST Special Publication 800-34, and the contractor's FedRAMP authorization. Every piece of it is billable, and it is billed continuously, across every federal population served: veterans, Medicare, Medicaid, CHIP, the federal workforce, military retirees.

I spent twenty-five years building and administering exactly these kinds of systems, including large-scale data-center and disaster-recovery operations. I have been on the floor for real cutover tests, the kind that move an operation to a facility hundreds of miles away and find out whether it actually holds. I raise this not as a guess, but because I know what these obligations cost, what it takes to maintain them for real, and what it looks like when they are maintained only on paper.

These are the specific, expensive, verifiable things a contract at this scale is supposed to require, and every one of them is a question with a factual answer someone can go and check:

Whether the alternate data facility is a genuinely separate, dedicated facility or a shared cage in a commercial colocation site, and whether the contract permits sharing at all given the data it holds.

Whether the site meets the physical, power, cooling, and communications specifications the contract calls for, or a fraction of them.

Whether the cross-country network capacity between primary and alternate sites actually exists at the contracted level, or terminates in something a fraction of the size.

Whether continuity and restoration are actually tested, on what schedule, and whether those tests are documented and passed, or documented and skipped.

Whether program data that is legally required to stay separated, across veterans, Medicare, Medicaid, CHIP, the federal workforce, and military retirees, is in fact separated, or consolidated in ways the contracts and the law do not permit.

Whether the HIPAA and equivalent operational safeguards that apply to this data are maintained in practice, not just on paper.

Standing up a genuine, tested alternate facility, with real separation and real cross-country capacity, is one of the largest cost centers an operation like this carries, and it generates no revenue. It exists only to sit ready. Every item above is separately billable, and none of it is done for free. That is precisely the kind of obligation that is expensive to meet and invisible to neglect.

Here is the reasoning I would ask you to weigh, and then hand to your own experts.

This company's willingness to alter records, and to misrepresent the state of its systems, is not something I am speculating about. It is documented in my own file, and it is documented in a written misrepresentation to a United States Senator about whether my portal was locked. That is established.

The fraud that has been publicly reported occurs in the visible places, in billing, in claims, in the parts of the operation where people are affected and where harm can be seen and reported. Infrastructure is not like that. A data center does not file a complaint. A neglected continuity obligation does not generate a headline. Hardware does not scream out for help. The parts of this operation that human beings can see and report are the parts that get attention. The structural and infrastructure obligations do not have anyone standing over them who is prepared to risk a job to report a shortfall.

So the question is simple, and it is a fair one. An organization demonstrably willing to cut corners where people are watching invites the question of what is being done where no one is. I am not answering that question. I have no way to. I am identifying it as a place where a party that has already shown willingness to alter records at the claim level should be asked to demonstrate, not attest, that its federally required, federally billed data-integrity and continuity obligations are genuinely intact and genuinely tested.

The distinction between attesting and demonstrating is the whole of it. They attested to a Senator that my portal was not locked. My records show it was. That is the difference between what this organization says about its systems and what its systems actually show. On everything federally required and continuously billed, I would ask that they be made to show it working rather than swear it works.

I recognize I may be reaching further than I can see. I am telling you plainly that I am reaching, and why. When I followed this up the chain from my own altered records, I arrived at dollar signs, and I do not believe I am wrong to have arrived there. Talk to your own experts. Let them decide where it goes.

A NOTE ON WHAT THIS MIGHT BE

I am not going to tell you what statute this fits. That is your work and I know the difference between us.

But I would be dishonest if I laid all of this out and then pretended I could not see the shape of it.

An organization. Conduct sustained across years rather than a single act. Claims moving through the mail and across wires. Records altered after the fact. A false statement to a United States Senator, in writing, about whether that alteration was even possible. A federal regulation the company does not hold, invoked to keep a fraud complainant from learning anything.

I said the word RICO to a federal agent before I understood what I was holding. I said it because it was the only word I had for a pattern that did not look like one person making one mistake.

I am not asking you to agree. I am telling you that a man with no legal training looked at this and reached for that word, and I would rather say so plainly than pretend the thought never occurred to me.

If the pattern does not support it, you will know that in an hour and I will accept it. If it does, you already knew before I said anything.

THE SCALE THIS SITS AGAINST

I want to put my own case in proportion, because the proportion is the argument.

The documented fraud in my file runs to roughly twenty-five thousand dollars. One veteran. One dental course of treatment. Found by the patient, with no authority, no subpoena power, and no access beyond his own records.

UnitedHealth Group reported full-year 2025 revenue of \$447.6 billion, up twelve percent year over year. Optum alone reported \$270.6 billion. UnitedHealthcare's Medicare and Retirement segment reported \$171.3 billion. Its Community and State segment, which is Medicaid, reported \$94.4 billion. Published analysis of the company's own filings puts more than three-quarters of UnitedHealthcare's revenue as coming from taxpayer-funded government programs.

I am not drawing a conclusion from those numbers. I am placing them next to mine.

The smallest visible instance of this conduct cost twenty-five thousand dollars and was found by one patient looking at his own file. The same mechanism, operating across veterans, Medicare, Medicaid, CHIP, the federal workforce, and military retirees, sits inside an operation of that size. What the ratio suggests is not something I can prove and I am not asserting it. It is the reason I stopped believing this was about me, and the reason I brought it to you rather than to a malpractice attorney.

Figures above are from UnitedHealth Group's own 2025 results release and published analysis of its filings. All are checkable.

WHAT DOES NOT DEPEND ON ME

None of the following requires anyone to believe a word I say.

VA OIG has published figures on uncollected community care recoveries, on what the Program Integrity Tool pause cost per month while it was down, and on projected improper payments for dental procedures.

Separately, in 2026 Massachusetts sued UnitedHealthcare over Medicaid conduct, alleging a growth-at-all-costs strategy that encouraged coding members as sicker than they were, seeking one hundred million dollars subject to trebling under the state False Claims Act.

Different program. Different state. Same mechanism.

OTHER EFFECTS

There is a body of published reporting on Optum's VA-specific conduct and on UnitedHealth Group across every major line it covers. I use both names in this document, sometimes interchangeably, and I recognize that. It does not change the substance.

And if the VA could be used this way, the federal workforce is not a separate question. Same company, same billing infrastructure, same incentives, administering coverage for millions of federal employees. The question is not whether they are exposed. It is who has looked.

As I learned more, I recognized this was not about me.

The race to the bottom. The published reporting. The dollar figures. And my own records moving through a system while the fraud detection was dark.

Every report, every open secret, every action taken in front of people who could see it. And somehow I am the one holding bottom-up documentary proof. I lit a match to see what I was standing in, looked around, and found the room full of dynamite.

NOT AN ORGANIZATION YOU SHUT OFF

Optum and UnitedHealth Group employ a great many good people, and that is true across parts of this I have not even researched. The contracts with the VA were structured in ways that were, frankly, a joke, and the VA knew it and could do nothing about it. Around every terrible aspect of this are real people who want to help others, and I am certain many of them go home knowing they cannot fix what they see.

Mr. Poehling's case alone causes me pain to read.

QUESTIONS FOR COUNSEL

These are questions, not claims. I raise them because I have been asked what I think this is, and I would rather put my reasoning in front of you than have it sit unspoken.

The False Claims Act is the vehicle I understand. The billing is documented, the alterations are documented, and that is the part I can prove.

What I do not know how to characterize is the conduct that produced the billing. A contractor holds a position inside a federal program. Federal employees acted, over years, against a beneficiary of that program in ways that ran contrary to their duty. I cannot show you the instruction. I can show you the pattern and the effects.

The questions I would put to you:

Does an agreement to obstruct a lawful government function — the delivery of care to veterans — reach 18 U.S.C. Section 371, conspiracy to defraud the United States, given that it does not appear to require anything of value to have changed hands?

If providers were pressured through their livelihoods, does that fall within 18 U.S.C. Section 1951?

Does 18 U.S.C. Section 241 reach a scheme to injure a person in the exercise of an entitlement under a federal program? I raise it particularly because the method depended on a documented psychiatric diagnosis, and the diagnosis was the instrument.

Does the conduct on the VA side implicate 18 U.S.C. Section 242?

Do the portal lockout and what followed it touch 18 U.S.C. Sections 1512 or 1513?

I understand that every one of these turns on proving the instruction — who told whom, and when. That is the part I cannot reach. It is also, as I understand it, precisely what subpoena power exists to produce.

One further observation, offered as strategy rather than law. A number of people were drawn into this who did not want to be. I withdrew every complaint I filed against them before anyone asked me to, and I did it publicly. Whatever else that accomplishes, it means I am not the thing any of them has to be afraid of.

THE RELATOR

Nothing in this section is load-bearing for the findings above. It is here so that nothing about the person bringing this is a surprise.

WHO YOU ARE WORKING WITH

I did not ask for the family that shaped me and my younger brother. His effort to end himself worked. I was selected next — for being who I am, and because of a family trust and a recovery lawsuit. The funds were split between me, my niece, and my nephew. My share was twenty-five thousand dollars.

That was the number my life was worth.

I use no stealth. I hold no secrets for anyone, including myself. I intend to post my own seven-thousand-page VA medical record publicly. That is my armor. I am very good at absorbing pain and standing back up, and I have added some skills since.

If I were reading this, I would ask whether this man had family, friends, or any capacity for self-defense before the word rehab. No. I did not.

I walked into rehab on a broken foot, drunk, after a night of walking. Walked out for one last case of pancreatitis and back in for forty-five days. There is not a day since that I would trade for any day before it.

The record since: a 0.42 GPA in the 1990s, which no longer counts toward anything. Then an associate's, including spring of 2024 — twelve classes, thirty-six credits, eleven A's and one B. A bachelor's in information systems. A master's closing in fall 2026. Every average since rehab has run 3.6 or better.

I do not believe in luck. Coincidences happen. Everything else is intention.

The VA was used to influence me to end myself.

I want to be precise about what that did and did not do. I am not a common presentation of borderline personality disorder. No splitting. No self-harm. No self-deletion. I have never hated myself. I have never hated God, and He does not hate me. I intend to live past two hundred and watch what epigenetics does, because the IT staff upstairs is not waiting on me.

It took fifty-two years to reach the starting line. All of this changed me. Beyond that, I am in contact with the Centre for Cybercrime and Economic Crime at the University of Portsmouth about doctoral work on enforcement fragmentation — what happens when oversight of one large contractor is divided across many bodies and none holds the whole picture. USF and VR&E, the VA office funding my education, are aware of the direction.

THE CONTROL GROUP

There is a control group in this, and it is the cleanest answer to any suggestion that I am an unreliable narrator.

The same information, from the same man, went to two institutions.

USF received it and acted on it.

The VA had the same information, its own diagnosis of me in its own file, and a duty of care — and ran the other direction for years.

USF proves the material was real and actionable.

WHAT I AM DISCLOSING ON DAY ONE

Three things, surfaced by me now rather than found later.

I posted one document publicly on LinkedIn to attract qui tam counsel and to counter efforts already being made against me in the open. Early outreach also went out from my university email address, which is subject to Florida public records law. Both are public exposure and both are on the table.

During the worst years I sent a large volume of email from personal accounts, with correlating VA secure messages on the same nights. I am the custodian of all of it. Nothing is being withheld, and I would rather you have it from me as a measure of the pressure being applied than have it characterized as something else.

And for completeness, because opposing counsel will look: some years ago I was arrested on a charge of battery on a witness following a domestic incident. The matter never went to trial and I was never convicted. It was diverted to Veterans Treatment Court, where I completed the program and was released from probation early for compliance. The record was expunged under Florida law. I raise it not because I am required to, but because my practice with my own counsel is full disclosure.

I have made mistakes getting here. I reached wrong conclusions and I corrected them.

For a long time I asked how these doctors could live with themselves knowing what they were doing. Then I learned they were coerced. Six years of it — while making decisions about houses, about their children's educations. They were never meant to be dragged into my childhood damage or into a fight I was uncovering while rebuilding my own existence under fire.

IT'S NOT ABOUT ME

However this happened — and I got here by an unusual route — I endured it, got back up, and rebuilt my life under fire before I understood any of what was being done to me.

What that combination left me with is useful here. Stoic borderline to Sigma INFJ, clinical and non-clinical terms that both happen to work. What it means in practice is that I can live with uncertainty, which no narcissist can. That sounds small. It is not. It is the thing that let me hold an incomplete picture for years without forcing it into a conclusion, and it is the reason the conclusions I did reach held up. The pattern recognition and the technical background did not hurt either.

My own past plus this combination made me immune to Tuesdays.

There was a point where I believed pursuing this might cost me a financial recovery. I decided: so what. This gets done.

I would do this for nothing. But doing it for nothing does not let your firms recover what this will cost you, and I leave that side of it entirely to your judgment.

I am not being naive when I say that ending the race to the bottom, and ending this kind of fraud, would be good for a great many people. Those two things are worth more than what happened to me.

The VA and the American people are behind me on this. I am blessed to have the life I have.

Today this is about others.

Steven Barden

Nerfing Tuesdays Since Rehab

REFERENCES

These support the public matters cited above. They are provided so the points that do not depend on me can be checked without taking my word for anything. Matters still in active litigation are allegations, not findings.

Medicare, DOJ intervention and the roughly two-billion-dollar figure: United States ex rel. Poehling v. UnitedHealth Group; see also KFF Health News reporting on the special master ruling.

Medicare, nH Predict post-acute care denials and the ninety percent error-rate allegation: class action filed 2023, U.S. District Court, District of Minnesota (naviHealth / Optum); STAT News investigation; CBS News reporting.

Medicaid, the one-hundred-million-dollar Massachusetts action: Commonwealth of Massachusetts v. UnitedHealthcare, 2026, Office of the Attorney General; STAT News and Healthcare Dive reporting.

Children, duplicate CHIP and Medicaid coverage paid to managed care organizations, and unallowable payments for beneficiaries with multiple identifiers: U.S. Government Accountability Office and HHS Office of Inspector General reports.

Veterans, improper community care payments and uncollected recoveries: VA Office of Inspector General published reports.

All figures are drawn from public reporting and government publications and should be confirmed against the primary source before any filing.

THE RECORDS THEMSELVES

The three Optum record pulls at the center of this — the ones that do not match — are posted for you to retrieve and examine directly:

<https://github.com/StevenBarden/Barden-VA-Optum-Billing-Fraud/tree/main/02-evidentiary-pdf>

These are the OCR working copies, made searchable. I retain the original, unaltered exports. If any question is ever raised about the integrity of a posted file, I can produce the untouched original for comparison.

If access requires a password, it is: NerfingTuesdays

Everything else — the full records, the secure messages spanning years, the congressional correspondence, and the decade of VA file — is organized and available immediately on request.